A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

20 May 2026

Dr Tania Siddiqui
Canterbury Medical Clinic

Dear Dr Siddiqui,

Re: Mrs Lynette Bromley
DOB: 17/10/1975 Phone: 0409 197 510
Address: 25 Stanhope Grove, CAMBERWELL, VIC, 3124
Email: lynette_krstev@hotmail.com

Summary

- **Functional abdominal pain and bloating with significant symptomatic improvement on a prebiotic supplement.**
- **CT abdomen and serum markers (including IgG4 and tumour markers) have excluded pancreatic pathology.**
- **Incidental benign renal cysts requiring no further investigation.**
- **No action required by you.**

I had the pleasure of conducting a review with Lynette Bromley to discuss her ongoing symptoms and recent investigation results. I am pleased to report that the CT scan of her abdomen was entirely reassuring, with no evidence of sinister pathology. Her pancreas appears completely healthy, and although the scan noted a few minor scattered renal cysts, these are common age-related findings and do not require any specific follow-up.

Clinically, Lynette is doing much better. While she briefly trialled nizatidine following our last appointment, she elected to cease this after she felt it made her symptoms worse. Interestingly, she has since commenced a prebiotic supplement (Wellco Super Elixir) containing ingredients such as psyllium, guar gum, and magnesium. She reports that her bowel movements are softer and, more importantly, she has been essentially free of abdominal pain for several months. We discussed that the guar gum and magnesium likely assist with stool consistency and gas transit, which may be contributing to her current comfort. From a diagnostic perspective, we have now excluded significant structural or inflammatory issues. Her recent blood tests, including protein studies, inflammatory markers, and pancreatic tumour markers, were all normal. Furthermore, we specifically investigated the possibility of autoimmune pancreatitis, but both her IgG4 levels and CT imaging were negative for this condition.

Impression and Plan

Lynette's symptoms are functional in nature. Given she is currently well-controlled on her current supplement regimen, I have advised her to continue with this. We have discussed several options should her symptoms of pain or bloating recur in the future. If pain becomes a prominent feature again, we could consider a cycled course of nizatidine or, preferably, a low-dose neuromodulator such as nortriptyline to address visceral hypersensitivity.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
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Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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For her remaining symptoms of bloating and distension, I have provided a prescription for a two-week course of rifaximin. I have explained that this antibiotic can be effective in resetting the gut microbiota and is often helpful for bloating, even if its effect on pain is less predictable. I have advised her to have this filled at a compounding pharmacy to avoid unnecessary costs. At this stage, Lynette is happy to manage things as they are and will contact me for a further review only if her symptoms change or deteriorate.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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